Pharmacode position may change as per Supplier's m/c requirement &additional small pharma code may appear on the front / back panel

ALDRONA 70
P1531034

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SUMMARY OF PRODUCT CHARACTERISTICS

ALDRONA 70

Alendronic acid Tablets 70 mg

Rx Only

NAME OF THE MEDICINAL PRODUCT:	Alendronic Acid Tablets 70 mg
TRADE NAME OF THE PRODUCT:	ALDRONA 70
STENGTH:	70 mg
PHARMACEUTICAL FORM:	Tablet.

DESCRIPTION: White to off-white, oval shaped, biconvex, uncoated tablets debossed with 'F' on one side and '21' on the other side.

QUALITATIVE AND QUANTITATIVE COMPOSITION

Each uncoated tablet contains

Sodium alendronate Ph.Eur equivalent to Alendronic acid 70 mg.

CLINICAL PARTICULARS

Therapeutic indications

Alendronic acid is indicated for the treatment of osteoporosis in postmenopausal women.

For the treatment of osteoporosis, Alendronic acid increases bone mass and prevents fractures including those of the hip and spine (vertebral compression fractures). Osteoporosis may be confirmed by the finding of low bone mass or by the presence or history of osteoporotic fracture.

Alendronic acid is indicated for the treatment of osteoporosis in men to prevent fractures.

Posology and method of administration

The recommended dosage is one 70 mg tablet once weekly.

To permit adequate absorption of alendronic acid:

Alendronic acid must be taken at least 30 minutes before the first food, beverage, or medicinal product of the day with plain water only. Other beverages (including mineral water), food and some medicinal products are likely to reduce the absorption of alendronic acid (Refer Interaction with other medicinal products and other forms of interaction section).

To facilitate delivery to the stomach and thus reduce the potential for local and oesophageal irritation/adverse experiences (Refer Special warnings and precautions for use section):

- Alendronic acid should only be swallowed upon arising for the day with a full glass of water (not less than 200 ml or 7 fl.oz.).
- Patients should not chew the tablet or allow the tablet to dissolve in their mouths because of a potential for oropharyngeal ulceration.
- Patients should not lie down until after their first food of the day which should be at least 30 minutes after taking the tablet.
- Patients should not lie down for at least 30 minutes after taking alendronic acid.
- Alendronic acid should not be taken at bedtime or before arising for the day.
- Failure to follow these instructions may increase the risk of esophageal adverse experiences

Patients should receive supplemental calcium and vitamin D if dietary intake is inadequate (Refer Special warnings and precautions for use section).

Use in the elderly: In clinical studies there was no age-related difference in the efficacy or safety profiles of alendronic acid. Therefore no dosage adjustment is necessary for the elderly.

Use in renal impairment: No dosage adjustment is necessary for patients with GFR greater than 35 ml/min. Alendronic acid is not recommended for patients with renal impairment where GFR is less than 35 ml/min, due to lack of experience.

Method of administration

Oral

Contraindications

- Hypersensitivity to alendronic acid or to any of the excipients.
- Abnormalities of the oesophagus and other factors which delay oesophageal emptying such as stricture or achalasia.
- Inability to stand or sit upright for at least 30 minutes.
- Hypocalcaemia.

Special warnings and precautions for use

Alendronic acid can cause local irritation of the upper gastro-intestinal mucosa. Because there is a potential for worsening of the underlying disease, caution should be used when alendronic acid is given to patients with active upper gastro-intestinal problems, such as dysphagia, oesophageal disease, gastritis, duodenitis, ulcers, or with a recent history (within the previous year) of major gastro-intestinal disease such as peptic ulcer, or active gastro-intestinal bleeding, or surgery of the upper gastro-intestinal tract other than pyloroplasty.

Oesophageal reactions (sometimes severe and requiring hospitalisation), such as oesophagitis, oesophageal ulcers and oesophageal erosions, rarely followed by oesophageal stricture, may occur in patients receiving alendronic acid. Physicians should therefore be alert to any signs or symptoms signalling a possible oesophageal reaction and patients should be instructed to discontinue alendronic acid and seek medical attention if they develop symptoms of oesophageal irritation such as dysphagia, pain on swallowing or retrosternal pain, new or worsening heartburn.

The risk of severe oesophageal adverse experiences appears to be greater in patients who fail to take alendronic acid properly and/or who continue to take alendronic acid after developing symptoms suggestive of oesophageal irritation. It is very important that the full dosing instructions are provided to, and understood by the patient (Refer Posology and method of administration section). Patients should be informed that failure to follow these instructions may increase their risk of oesophageal problems.

Patients should be instructed that if they miss a dose of Alendronic Acid, they should take one tablet on the morning after they remember. They should not take two tablets on the same day but should return to taking one tablet once a week, as originally scheduled on their chosen day.

Alendronic acid is not recommended for patients with renal impairment where GFR is less than 35 ml/min (Refer Posology and method of administration section).

Causes of osteoporosis other than oestrogen deficiency and ageing should be considered.

Hypocalcaemia must be corrected before initiating therapy with alendronic acid (Refer Contraindications). Other disorders affecting mineral metabolism (such as vitamin D deficiency and hypoparathyroidism) should also be effectively treated. In patients with these conditions, serum calcium and symptoms of hypocalcaemia should be monitored during therapy with alendronic acid.

Due to positive effects of alendronic acid in increasing bone mineral, decreases in serum calcium and phosphate may occur. These are usually small and asymptomatic. However, symptomatic hypocalcaemia may rarely occur, which have occasionally been severe and often occurred in patients with predisposing conditions (e.g. hypoparathyroidism, vitamin D deficiency and calcium malabsorption). Ensuring adequate calcium and vitamin D intake is therefore particularly important in patients receiving glucocorticoids.

Osteonecrosis of the jaw, generally associated with tooth extraction and/or local infection (including osteomyelitis) may cause in patients with cancer receiving treatment regimens including primarily intravenously administered bisphosphonates. Many of these patients were also receiving chemotherapy and corticosteroids. Osteonecrosis of the jaw may also cause in patients with osteoporosis receiving oral bisphosphonates.

Osteonecrosis of the external auditory canal has been reported with bisphosphonates, mainly in association with long-term therapy. Possible risk factors for osteonecrosis of the external auditory canal include steroid use and chemotherapy and/or local risk factors such as infection or trauma. The possibility of osteonecrosis of the external auditory canal should be considered in patients receiving bisphosphonates who present with ear symptoms including chronic ear infections.

A dental examination with appropriate preventive dentistry should be considered prior to treatment with bisphosphonates in patients with concomitant risk factors (e.g. cancer, chemotherapy, radiotherapy, corticosteroids, poor oral hygiene, periodontal disease).

While on treatment, these patients should avoid invasive dental procedures if possible. For patients who develop osteonecrosis of the jaw while on bisphosphonate therapy, dental surgery may exacerbate the condition. For patients requiring dental procedures, there are no data available to suggest whether discontinuation of bisphosphonate treatment reduces the risk of osteonecrosis of the jaw.

Interaction with other medicinal products and other forms of interaction

If taken at the same time, it is likely that food and beverages (including mineral water), calcium supplements, antacids, and some oral medicinal products will interfere with absorption of alendronic acid. Therefore, patients must wait at least 30 minutes after taking alendronic acid before taking any other oral medicinal product (Refer Posology and method of administration and Pharmacokinetic properties).

No other interactions with medicinal products of clinical significance are anticipated.

Pregnancy and lactation

Use during pregnancy

There are no adequate data from the use of alendronic acid in pregnant women. Given the indication, alendronic acid should not be used during pregnancy.

Use during lactation

It is not known whether alendronic acid is excreted into human breast milk. Given the indication, alendronic acid should not be used by breast-feeding women.

A/s : 210 x 300 mm Booklet Size: 35 x 60 mm Black

AUROBINDO Packaging Development	Product Name: Aldrona 70	Component: Leaflet	Item Code: P1531034	Date & Time: 05.05.2022 & 5:50 PM
	Customer / Country: Malaysia	Version No.: 00	Reason Of Issue: Submission	Reviewed / Approved by
Team Leader: Kiran	Dimensions: 210 x 300 mm	No. of Colours : 01		
Initiator: Shirisha				
Artist: SCD	Pharmacode: 31034	31034		
Additional Information : Supersede code : P1515707				Sign / Date

Effects on ability to drive and use machines

No studies on the effects on the ability to drive and use machines have been performed.

Undesirable effects

Immune system disorders:

Rare: hypersensitivity reactions including urticaria and angioedema

Metabolism and nutrition disorders:

Rare: symptomatic hypocalcaemia, often in association with predisposing conditions. (Refer Special warnings and precautions for use section)

Nervous system disorders:

Common: headache

Eye disorders:

Rare: uveitis, scleritis, episcleritis

Gastrointestinal disorders:

Common: abdominal pain, dyspepsia, constipation, diarrhoea, flatulence, oesophageal ulcer*, dysphagia*, abdominal distension, acid regurgitation

Uncommon: nausea, vomiting, gastritis, oesophagitis*, oesophageal erosions*, melena

Rare: oesophageal stricture*, oropharyngeal ulceration*, upper gastrointestinal PUBs (perforation, ulcers, bleeding) (Refer Special warnings and precautions for use section)

* (Refer Posology and method of administration and Special warnings and precautions for use)

Skin and subcutaneous tissue disorders:

Uncommon: rash, pruritus, erythema

Rare: rash with photosensitivity

Very rare and isolated cases: isolated cases of severe skin reactions including Stevens-Johnson syndrome and toxic epidermal necrolysis

Musculoskeletal, connective tissue and bone disorders:

Common: musculoskeletal (bone, muscle or joint) pain

Rare: Osteonecrosis of the jaw is generally associated with tooth extraction and / or local infection (including osteomyelitis). Diagnosis of cancer, chemotherapy, radiotherapy, corticosteroids and poor oral hygiene are also deemed as risk factors; severe musculoskeletal (bone, muscle or joint) pain (Refer Special warnings and precautions for use section).

Very rare: Osteonecrosis of the external auditory canal (bisphosphonate class adverse reaction).

General disorders and administration site conditions:

Rare: transient symptoms as in an acute-phase response (myalgia, malaise and rarely, fever), typically in association with initiation of treatment.

Overdose

Hypocalcaemia, hypophosphataemia and upper gastro-intestinal adverse events, such as upset stomach, heartburn, oesophagitis, gastritis, or ulcer, may result from oral overdosage.

No specific information is available on the treatment of overdosage with alendronic acid. Milk or antacids should be given to bind alendronic acid. Owing to the risk of oesophageal irritation, vomiting should not be induced and the patient should remain fully upright.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic properties

The active ingredient of Alendronate sodium, is a bisphosphonate that acts as a potent specific inhibitor of osteoclats-mediated bone resorption. Bisphosphonates are synthetic analogs of pyrophosphate that binds to the hydroxyapatite found in the bone and specifically inhibits the activity of osteoclats, the bone-resorbing cells. Alendronate inhibits osteoclastic bone resorption with no direct effect on bone formation, although the latter process is ultimately reduced because bone resorption and formations are coupled during bone turn over.

Pharmacokinetic properties

Absorption

Relative to an intravenous reference dose, the oral mean bioavailability of alendronic acid in women was 0.64% for doses ranging from 5 to 70 mg when administered after an overnight fast and two hours before a standardised breakfast. Bioavailability was decreased similarly to an estimated 0.46% and 0.39% when alendronic acid was administered one hour or half an hour before a standardised breakfast.

Distribution

The mean steady-state volume of distribution, exclusive of bone, is at least 28 litres in humans. Concentrations of drug in plasma following therapeutic oral doses are too low for analytical detection (<5 ng/ml). Protein binding in human plasma is approximately 78%.

Biotransformation

There is no evidence that alendronic acid is metabolised in animals or humans.

Elimination

Following a single intravenous dose of [^{14}C]alendronic acid, approximately 50% of the radioactivity was excreted in the urine within 72 hours and little or no radioactivity was recovered in the faeces. The terminal half-life in humans is estimated to exceed ten years, reflecting release of alendronic acid from the skeleton.

PHARMACEUTICAL PARTICULARS

List of excipients

Cellulose, Microcrystalline

Maize Starch

Sodium Starch Glycolate (Type A)

Povidone

Magnesium stearate

Incompatibilities

Not applicable.

Shelf life

2 years.

Special precautions for storage

Store below 30°C.

Nature and contents of container

Blister of 4 tablets.

Manufactured By

AUROBINDO

Aurobindo Pharma Ltd.,
Unit III, Survey No. 313 & 314,
Bachupally, Bachupally Mandal,
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